Enlightenment, Romanticism, and Reform

Philippe Pinel

(1745–1826)

A Treatise on Insanity

(1801)

In overturning the old, feudal order, the French Revolution (1789–1799) opened up leadership positions to a new class and generation of eager men and women schooled in Enlightenment thought. One such individual was the physician Philippe Pinel. Born into a family of doctors, Pinel had long criticized the traditional Paris Faculty of Medicine for being insular and elitist, and he had been unafraid of expressing his reformist ideas and leanings. After the revolution, as the new government began reorganizing hospitals, poorhouses, prisons, and schools, Pinel was recruited, in 1792, to serve as chief physician at the Bicêtre Hospital in Paris, then asked a few years later to become director of the Salpêtrière asylum. In both facilities, he set about establishing what contemporaries were calling "the moral treatment." As the famous painting by Charles Muller shows, the changes Pinel instituted at Bicêtre were heralded as an emblem of enlightened Reason's triumph over a backward Old Regime.

Nothing has more contributed to the rapid improvement of modern natural history, than the spirit of minute and accurate observation which has

Philippe Pinel has the irons removed from the insane at Bicêtre. Mural by Charles Louis Muller (1815–1892). Académie de Médecine, Paris, France. © Bridgeman-Giraudon/Art Resource, NY.

distinguished its votaries. The habit of analytical investigation, thus adopted, has induced an accuracy of expression and a propriety of classification, which have themselves, in no small degree, contributed to the advancement of natural knowledge. Convinced of the essential of the same means in the illustration of a subject so new and so difficult as that of the present work, it will be seen that I have availed myself of their application, in all or most of the instances of this most calamitous disease, which occurred in my practice at the Asylum de Bicêtre. On my entrance upon the duties of that hospital, every thing presented to me the appearance of chaos and confusion. Some of my unfortunate patients laboured under the horrors of a most gloomy and desponding melancholy. Others were furious, and subject to the influence of a perpetual delirium. Some appeared to possess a correct judgment upon most subjects, but were occasionally agitated by violent sallies of maniacal fury; while those of another class were sunk into a state of stupid ideotism and imbecility. Symptoms so different, and all comprehended under the general title of insanity, required, on my part, much study and discrimination; and to secure order in the establishment and success to the practice, I determined upon adopting such a variety of measures, both as to discipline and treatment, as my patients required, and my limited opportunity permitted. From systems of nosology, I had little assistance to expect; since the arbitrary distributions of Sauvages and Cullen were better calculated to

impress the conviction of their insufficiency than to simplify my labour. I, therefore, resolved to adopt that method of investigation which has invariably succeeded in all the departments of natural history, viz. to notice successively every fact, without any other object than that of collecting materials for future use; and to endeavour, as far as possible, to divest myself of the influence, both of my own prepossessions and the authority of others. With this view, I first of all took a general statement of the symptoms of my patients. To ascertain their characteristic peculiarities, the above survey was followed by cautious and repeated examinations into the condition of the individuals. All our new cases were entered at great length upon the journals of the house. Due attention was paid to the changes of the seasons and the weather, and their respective influences upon the patients were minutely noticed. Having a peculiar attachment for the more general method of descriptive history, I did not confine myself to any exclusive mode of arranging my observations, nor to any one system of nosography. The facts which I have thus collected are now submitted to the consideration of the public, in the form of a regular treatise.

Few subjects in medicine are so intimately connected with the history and philosophy of the human mind as insanity. There are still fewer, where there are so many errors to rectify, and so many prejudices to remove. Derangement of the understanding is generally considered as an effect of an organic lesion of the brain, consequently as incurable; a supposition that is, in a great number of instances, contrary to anatomical fact. Public asylums for maniacs have been regarded as places of confinement for such of its members as are become dangerous to the peace of society. The managers of those institutions, who are frequently men of little knowledge and less humanity, have been permitted to exercise towards their innocent prisoners a most arbitrary system of cruelty and violence; while experience affords ample and daily proofs of the happier effect of a mild, conciliating treatment, rendered effective by steady and dispassionate firmness. Availing themselves of this consideration, many empirics have erected establishments for the reception of lunatics, and have practiced this very delicate branch of the healing heart with singular reputation. A great number of cures have undoubtedly been effected by those base born children of the profession; but, as might be expected, they have not in any degree contributed to the advancement of science by any valuable writings. It is on the other hand to

be lamented, that regular physicians have indulged in a blind routine of inefficient treatment, and have allowed themselves to be confined within the fairy circle of antiphlogisticism, and by that means to be diverted from the more important management of the mind. Thus, too generally, has the philosophy of this disease, by which I mean the history of its symptoms, of its progress, of its varieties, and of its treatment in and out of hospitals, been most strangely neglected.

. . . The successful application of moral regimen exclusively, gives weight to the supposition, that, in a majority of instances, there is no organic lesion of the brain nor of the cranium. In order however to ascertain the species, and to establish a nosology of insanity, so far as it depends upon physical derangement, I have omitted no opportunities of examination after death. . . . By these and other means, which will be developed in the sequel, I have been enabled to introduce a degree of method into the service of the hospital, and to class my patients in a great measure according to the varieties and inveteracy of their complaints. . . .

Periodical Insanity Independent of the Influence of the Seasons

. . . From a general examination of the patients, at the Asylum of Bicêtre, in the second year of the republic, which was undertaken for the purpose of ascertaining the relative number of each variety of the disease; it appeared, that, out of two hundred maniacs, there were fifty-two of the class subject to paroxysms of insanity at irregular periods; and only six, whose periods of accession observed a regular intermission. . . . I shall be excused, if I mention three more cases, whose paroxysms invariably returned after an interval of eighteen months, and lasted precisely six months. The peculiar character of those unfortunate cases consisted in a few but well marked circumstances. Their ideas were clear and connected;—as they indulged in no extravagances of fancy;—they answered with great pertinence and precision to the questions that were proposed to them: but they were under the dominion of a most ungovernable fury, and of a thirst equally ungovernable for deeds of blood. In the mean time, they were fully aware of their horrid propensity, but absolutely incapable, without coercive assistance, of suppressing the atrocious impulse. How are we to reconcile these facts to the opinion which Locke and Condillac entertained with regard to the nature of insanity, which they made to consist exclusively in a disposition to

associate ideas naturally incompatible, and to mistake ideas thus associated for real truths?

The Character of Maniacal Paroxysms Not Depending upon the Nature of the Exciting Causes, but upon the Constitution

. . . I cannot here avoid giving my most decided suffrage in favour of the moral qualities of maniacs. I have no where met, excepting in romances, with fonder husbands, more affectionate parents, more impassioned lovers, more pure and exalted patriots, than in the lunatic asylum, during their intervals of calmness and reason. A man of sensibility may go there every day of his life, and witness scenes of indescribable tenderness associated to a most estimable virtue.

Maniacal Paroxysms Characterised by a High Degree of Physical and Mental Energy

It is to be hoped, that the science of medicine will one day proscribe the very vague and inaccurate expression of "images traced in the brain, the unequal determination of blood into different parts of this viscus, the irregular movements of the animal spirits," &c. expressions which are to be met with in the best writings that have appeared on the human understanding, but which do not accord with the origin, the causes, and the history of insanity. The nervous excitement, which characterises the greatest number of cases, affects not the system physically by increasing muscular power and action only, but likewise the mind, by exciting a consciousness of supreme importance and irresistible strength. Entertaining a high opinion of his capacity of resistance, a maniac often indulges in the most extravagant flights of fancy and caprice; and, upon attempts being made to repress or coerce him, aims furious blows at his keeper, and wages war against as many of the servants or attendants as he supposes he can well master. If met, however, by a force evidently and convincingly superior, he submits without opposition or violence. This is a great and invaluable secret in the management of well regulated hospitals. I have known it prevent many fatal accidents, and contribute greatly toward the cure of insanity. I have, however, seen the nervous excitement in question, in some few instances, become extremely obstinate and incoercible.

The Variety and Profundity of Knowledge Requisite on the Part of the Physician, in Order to Secure Success in the Treatment of Insanity

The time, perhaps, is at length arrived when medicine in France, now liberated from the fetters imposed upon it, by the prejudices of custom, by interested ambition, by its association with religious institutions, and by the discredit in which it has been held in the public estimation, will be able to assume its proper dignity, to establish its theories on facts alone, to generalise those facts, and to maintain its level with the other departments of natural history. The principles of free enquiry, which the revolution has incorporated with our national politics, have opened a wide field to the energies of medical philosophy. But, it is chiefly in great hospitals and asylums, that those advantages will be immediately felt, from the opportunities which are there afforded of making a great number of observations, experiments, and comparisons. . . .

The Author's Inducements to Study the Principles of Moral Treatment

About that time I was engaged to attend, in a professional capacity, at an asylum, where I made observations upon this disease [insanity] for five successive years. My opportunities for the application of moral remedies, were, however, not numerous. Having no part of the management of the interior police of that institution, I had little or no influence over its servants. The person who was at the head of the establishment, had no interest in the cure of his wealthy patients, and he often, unequivocally, betrayed a desire, that every remedy should fail. At other times, he placed exclusive confidence in the utility of bathing or in the efficacy of petty and frivolous recipes. The administration of the civil hospitals, in Paris, opened to me in the second year of the republic a wide field of research, by my nomination to the office of chief physician to the national Asylum de Bicêtre, which I continued to fill for two years. In order, in some degree, to make up for the local disadvantages of the hospital, and the numerous inconveniences which arose from the instability and successive changes of administration, I determined to turn my attention, almost exclusively, to the subject of moral treatment. The hall and the passages of the hospital were much confined, and so arranged as to render the cold of winter and the heat of summer equally intolerable and injurious. The chambers were exceedingly

small and inconvenient. Baths we had none, though I made repeated applications for them; nor had we extensive liberties for walking, gardening or other exercises. So destitute of accommodation, we found it impossible to class our patients according to the varieties and degrees of their respective maladies. On the other hand, the gentleman, to whom was committed the chief management of the hospital, exercised towards all that were placed under his protection, the vigilance of a kind and affectionate parent.* Accustomed to reflect, and possessed of great experience, he was not deficient either in the knowledge or execution of the duties of his office. He never lost sight of the principles of a most genuine philanthropy. He paid great attention to the diet of the house, and left no opportunity for murmur or discontent on the part of the fastidious. He exercised a strict discipline over the conduct of the domestics, and punished, with severity, every instance of ill treatment, and every act of violence, of which they were guilty towards those whom it was merely their duty to serve. He was both esteemed and feared by every maniac; for he was mild, and at the same time inflexibly firm. In a word, he was master of every branch of his art, from its simplest to its most complicated principles. Thus was I introduced to a man, whose friendship was an invaluable acquisition to me. . . .

The Advantages of Restraint upon the Imagination of Maniacs Illustrated

A young religious enthusiast, who was exceedingly affected by the abolition of the catholic religion in France, became insane. After the usual treatment at the Hotel Dieu, he was transferred to the Asylum de Bicêtre.† His misanthropy was not to be equaled. His thoughts dwelled perpetually upon the torments of the other world; from which he founded his only chance of escaping, upon conscientious adoption of the abstinences and mortifications of the ancient anchorites. At length, he refused nourishment altogether; and on the fourth day after that unfortunate resolution was formed, a state of langour succeeded, which excited considerable apprehension for

*Editor's note: This was Jean-Baptiste Pussin (1745–1811), a tanner who himself had been a scrofula patient in the Bicêtre Asylum in 1771 and was later cured. He was thereafter employed by the hospital, and in 1784, he became the superintendent of the ward for incurable mental patients.

†Editor's note: Hôtel-Dieu is one of the oldest hospitals in Paris.

his life. Kind remonstrances and pressing invitations proved equally ineffectual. He repelled, with rudeness, the services of the attendants, rejected, with the utmost pernacity, some soup that was placed before him, and demolished his bed (which was of straw) in order that he might lie upon the boards. How was such a perverse train of ideas to be stemmed or counteracted? The excitement of terror presented itself as the only resource. For this purpose, Citizen Pussin appeared one night at the door of his chamber, and, with fire darting from his eyes, and thunder in his voice, commanded a group of domestics, who were armed with strong and loudly clanking chains, to do their duty. But the ceremony was artfully suspended;—the soup was placed before the maniac, and strict orders were left him to eat it in the course of the night, on pains of the severest punishment. He was left to his own reflections. The night was spent (as he afterwards informed me) in a state of most distressing hesitation, whether to incur the present punishment, or the distant but still more dreadful torments of the world to come. After an internal struggle of many hours, the idea of the present evil gained the ascendancy; and he determined to take the soup. From that time he submitted, without difficulty, to a restorative system of regimen. His sleep and strength gradually returned; his reason recovered its empire; and, after the manner above related, he escaped certain death. It was during his convalescence, that he mentioned to me the perplexities and agitations which he endured during the night of the experiment.

A Happy Expedient Employed in the Cure of a Mechanician

A celebrated watchmaker, at Paris, was infatuated with the chimera of perpetual motion, and to effect this discovery, he set to work with indefatigable ardour. From unremitting attention to the object of his enthusiasm coinciding with the influence of revolutionary disturbances, his imagination was greatly heated, his sleep was interrupted, and, at length, a complete mental derangement of the understanding took place. His case was marked by a most whimsical illusion of the imagination. He fancied that he had lost his head on the scaffold; that it had been thrown promiscuously among the heads of many other victims; that the judges, having repented of their cruel sentences, had ordered those heads to be restored to their respective owners, and placed upon their respective shoulders; but that, in consequence of an unfortunate mistake, the gentlemen, who had the management of that

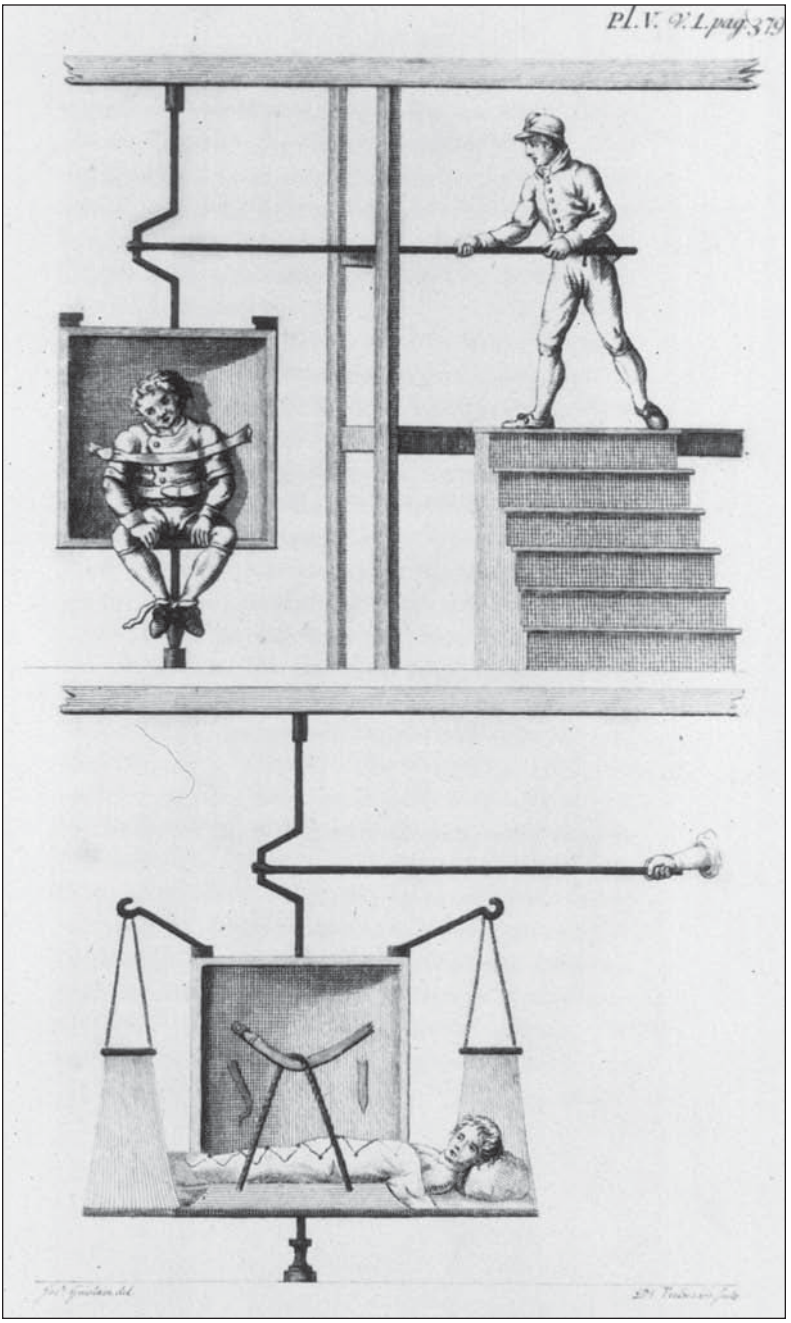
business, had placed upon his shoulder the head of one of his unhappy companions. The idea of this whimsical exchange of his head, occupied his thoughts night and day; which determined his relations to send him to the Hotel Dieu. Thence he was transferred to the Asylum de Bicêtre. Nothing could equal the extravagant overflowings of his heated brain. He sung, cried, or danced incessantly; and, as there appeared no propensity in him to commit acts of violence or disturbance, he was always allowed to go about the hospital without control, in order to expend, by evaporation, the effervescent excess of his spirits. "Look at these teeth," he constantly cried,—"Mine were exceedingly handsome;—these are rotten and decayed. My mouth was sound and healthy: this is foul and diseased. What difference between this hair and that of my own head. . . ."

A keen and unanswerable stroke of pleasantry seemed best adapted to correct this fantastic whim. Another convalescent of a gay and facetious humour, instructed in the part he should play in this comedy, adroitly turned the conversation to the subject of the famous miracle of Saint Denis.* Our mechanician strongly maintained the possibility of the fact, and sought to confirm it by an application of his own case. The other set up a loud laugh, and replied with a tone of the keenest ridicule: "Madman as thou art, how could Saint Denis kiss his own head? Was it with his heels?" This equally unexpected and unanswerable retort, forcibly struck the maniac. He retired confused amidst the peals of laughter, which were provoked at his expense, and never afterwards mentioned the exchange of his head. Close attention to his trade for some months, completed the restoration of his intellect. He was sent to his family in perfect health; and has, now for more than five years, pursued his business without a return of his complaint.

Maniacal Fury to be Repressed; but Not by Cruel Treatment

The lesions of the human intellect simply, embrace but part of the object of the present treatise. The active faculties of the mind are not less subject to serious lesions and changes, nor less deserving of ample consideration. The

*Editor's note: Saint Denis was a third-century Christian martyr and bishop of Paris. As legend has it, after Saint Denis was beheaded by opponents, he picked up his own head and continued to preach for several miles.



Two views of circulating swings for treating insanity. Devices like these were often used to calm acutely agitated patients. From William Saunders Hallaran, *Practical Observations of the Causes and Cure of Insanity* (Cork: Edwards and Savage, 1818), 95. Courtesy of the National Library of Medicine, Bethesda, MD.

diseased affections of the will—excessive or defective emotions, passion, &c. whether intermittent or continued, are sometimes associated with lesions of the intellect. At other times, however, the understanding is perfectly free in every department of its exercise. In all cases of excessive excitement of the passion, a method of treatment, simple enough in application, but highly calculated to render the disease incurable, has been adopted from time immemorial;—that of abandoning the patient to his melancholy fate, as an untameable being, to be immured in solitary durance, loaded with chains, or otherwise treated with extreme severity, until the natural close of a life so wretched shall rescue him from his misery, and convey him from the cells of the mad-house to the chambers of the grave. But this treatment convenient indeed to a governor, more remarkable for his indolence and ignorance than for his prudence or humanity, deserves, at the present day, to be held up to public execration, and classed with the other prejudices which have degraded the character and pretensions of the human species. To allow every maniac all the latitude of personal liberty consistent with safety; to proportion the degree of coercion to the demands upon it from his extravagance of behaviour; to use mildness of manners or firmness as occasion may require,—the bland arts of conciliation, or the tone of irresistible authority pronouncing an irreversible mandate, and to proscribe, most absolutely, all violence and ill treatment on the part of the domestics, are laws of fundamental importance, and essential to the prudent and successful management of all lunatic institutions. But how many great qualities, both of mind and body, it is necessary that the governor should possess, in order to meet the endless difficulties and exigencies of so responsible a situation!

From Philippe Pinel, *A Treatise on Insanity*, translated from the French by D. D. Davis (Sheffield: W. Todd, 1806), 1–6, 13–14, 16, 27–28, 52–54, 61–63, 68–72, 82–83.

Johann Christian August Heinroth
(1773–1843)
Textbook of Disturbances of Mental Life
(1818)

The German physician J.C.A. Heinroth was among the first generation of so-called mental doctors (*psychische Ärzte*), a group of physicians who combined the tradition of mad-doctoring with academic learning. Having studied theology for a time, he then took up medicine, working in military hospitals and a workhouse before becoming professor of psychotherapy (the first in Europe) at the University of Leipzig in 1811. Along with a host of other prominent asylum directors and physicians in Germany at the time, Heinroth was influenced by the intellectual movement of Romanticism, which emphasized values such as reason, the unity of mind and body, spiritual growth, and moral awakening. For him, madness was the result of an alienation from nature, a condition he believed was inherent to modern life. Although rarely acknowledged today, Heinroth and the other Romantic mental doctors were enormously influential in the nineteenth century, responsible for coining the term *psychosomatic* and developing the concept of the conflicted personality—notions that would later be taken up by Sigmund Freud. The following excerpt from Heinroth's textbook explains his general understanding of mental health and illness as well as the kind of role he believed the mental doctors needed to play in treating those in their care.

The Concept of Disturbed Mental Life or Disturbances of the Soul

The development of human life through all its ages may be considered as a journey made at a measured pace and aimed at the highest consciousness or life of reason; nay, one is forced to this conclusion or else to consider that man, with all the tendencies and forces which determine his life, is a creature which spends its existence in perpetual self-contradiction. The free creative force in man, his imagination, at first resembles the still amorphous

sap of plants which rises by way of roots, stem, branches, and leaves, is purified and transformed into flowers and fruit. The mental life of a child is sensual, and the imagination of the child exercises itself in the sensual world in play. The play urge is the child's expression of love. The mental life of the age of youth is also wholly dedicated to the imagination and concentrates the entire activity of the creative force on one point, on one object, viz., beauty; the beauty urge is the expression of love in youth. Not necessity but love is also the mother of the arts which originate from the individual man and from the human race, inasmuch as the individual and society live and love in the way of the young. The mental life of the mature age extends all the accumulated and complete activities of the creative force into the broad spheres of life with the aid of reason, and it is the business of this age to understand and to bring order, to enlighten and to control, and to stand free and independent through reason or at least to strive for freedom and independence. The urge for freedom is the expression of love of mental life at its zenith. . . .

This is the way man ought to grow. We learn this through faithful observation of his developing urge for growth in a regular determined form. But man is not a plant, and natural necessity is not his all-powerful master. Even though his conscience, his supreme law, affects him with all the severity of a natural necessity, he is nevertheless free not to obey it. Thus he is the first and the only creature on this earth who is a free agent. . . .

The Divine intentions in man are frustrated by man himself in many different ways. The way to the highest development in world-consciousness and in self-consciousness leads through the senses, imagination, and reason, but human life must not become arrested at the lower stages of development and refuse the Divine summons to proceed to higher stages. The man who scorns this repeated summons and is content with and stays only in the non-Divine existence and life will become enslaved by the non-Divine and lose his free will; this loss will not be direct or immediate, but the only possible truly free condition of life, and with it the feeling of pure satisfaction and joy, will be lost to him. A prey to passions, madness, and vice, the creative processes will be impeded, halted, and forced back in many different ways. Thus, by observing such a disturbed process of the inner organization that should have served to sustain the complete life, that is, the free life, we arrive at the concept of a disturbed mental life, or, in short, disturbance of the soul.

This concept is as yet very general, and no definite meaning has so far been assigned to it. It means nothing more than a mental life impeded in some way in its normal growth. Thus, any diseased condition could be denoted as mental disturbance. However, it must be borne in mind that passion, madness, and even vice often assist the soul of a man who, admonished by the voice of his conscience heard through the dim fog of his condition, may gather his forces, break his chains, and rise to a freer, higher plan and pursue good with a greater will. Furthermore, in any soul which still retains its free will, that is, at least potentially free, and which is enslaved by some but not all relationships of life (for good seed often bears fruit in the midst of weeds), the condition of the disturbance, the whole interference with the inner life, is neither complete nor exhaustive. Therefore the concept of disturbance of the soul must be understood more precisely as a total halt, a total standstill, or else an innate desire of the creative force, which was originally intended to produce the highest development, for the opposite, that is, for self-destruction, and must be restricted to cases in which such signs are distinctly evident.

In this condition the free will exists no more and is replaced by complete and permanent loss of freedom. This condition prevails in diseases commonly known as mental breakdown, aberrations of reason, madness, disease of temperament, mental diseases in general, etc. All these diseases, however, much as their external manifestations may differ, have this one feature in common, namely, that not only is there no freedom but not even the capacity to regain freedom. The world-consciousness and the self-consciousness are to a greater or lesser extent disturbed, confused, or wholly extinct, while there is no room for the reasoned consciousness, since free will, which is the receptacle of this consciousness, has died. Thus, individuals in this condition exist no longer in the human domain, which is the domain of freedom, but follow the coercion of internal and external natural necessity. Rather than resembling animals, which are led by a wholesome instinct, they resemble machines and are maintained by vital laws in bodily life alone.

The Concept of Doctor of the Psyche

If we assume that it is possible to cure mental disturbances, or at least to cure some of them under certain conditions, there arises the following

question. Since it is the degenerate mental life which must be led back to normal, since it is the humanly healthy condition which must be restored, would this be the task of a doctor? or perhaps of a cleric? of a philosopher? or of an educator? There are arguments which speak in favor of each of these four viewpoints, and each of these professions is at least apparently entitled to take possession of this curative task. . . .

Since we are speaking of medical art and science, we should think that nobody but a doctor should have a right to make mental disturbance the object of his studies and treatment. Indeed, doctors have claimed this right in their compendia and practice. . . . Nevertheless, since we are claiming that mental disturbances are the opposite of human health and since this claim is not arbitrary but stems from a faithful observation of human nature, we must separate the entire sphere of manifestations from the forms of illness which have symptoms the doctors are accustomed to diagnose. We must transfer them to another domain, the domain of mental life, with which doctors (since they are only familiar with bodily nature) are not familiar with regard to both the recognition of the disease and its treatment. The medical studies to be indicated below testify to this complete ignorance; furthermore, the point of view and the sphere of activity for which doctors are trained and prepared at high schools of learning and at the sickbed are totally different from the legitimate and true ones given in the present textbook. Accordingly, since doctors are pupils and adepts of medicinal art and science in the field of disturbed bodily lives only, they are not directly and immediately suited, at least not in the present condition of both education and experience as practicants, to carry out the business of healing the psyche.

The clerics, as the recognized shepherds of the soul, are just as unfit for the tasks, owing to their point of view and the training and direction they have received. For their field of activity is the moral nature of man for as long as it exists and not after it has died or at least temporarily disappeared. Their business, their profession, is thus concerned with a sphere which is quite different from the one which the doctor of the soul must be familiar. Philosophers, especially psychologists, have at times ventured into the sphere of disturbed mental life, at least theoretically, but they cannot accomplish anything, even in the theory of mental disturbances alone, unless they apply themselves to a faithful observation of nature. This has

not yet happened, as will be demonstrated later. Since their activity is confined to the writing desk, nothing in the nature of practical work can be expected of them, whereas the purpose of medicine of the psyche is precisely to take action in order to teach the art of guiding the disturbed mental life back to normal.

This science and art has much more in common with the art of an educator, even doctors agree that curing mentally disturbed individuals is at the same time a reeducation. But all will consent, at least, that this science and art was not invented by educators, even if it already existed; and educators, just like clerics, are at present trained and prepared to deal with free human force, but not to restore a freedom which has been lost. . . .

However, these requirements, or at least some of them, are such as can never be met by an educator or by a cleric or by a philosopher. For the doctor of the psyche must first be a physician, in the full ordinary meaning of the word. He must be learned in the medical traditions and versed in medical practice, partly because mental disturbances are often accompanied by bodily disturbances which they excite, maintain, and modify, partly because in very many instances it is possible to influence the mentally disturbed only through their bodies. It must therefore be concluded that the doctor of the psyche must indeed come forth from the class of physicians. We are purposely saying come forth, for he must not remain in this class, firstly because this class is sufficiently occupied in its own field, whereas the field of soul medicine is so large that the forces of an active man are fully engaged therein; secondly because a doctor of the psyche must undergo special training and must go in a direction which is altogether different from that of the doctor of the body.

For whoever takes upon himself to be a doctor of the psyche must be specially schooled by the psychologist, by the cleric, and by the educator; or rather, he must develop in himself the gift for psychological observation, must adopt a religious point of view, and must himself attempt to live the life of a cleric, or such a life as a pious man would live, that is, to lead a life of reason, or, in the words of the Holy Writ, a life in Christ, or must walk in light, all of which is the same thing. Finally, he must become proficient in the methods of the educator, transform them to his own ends, and carry them over into his own sphere. . . .

This is what a doctor of the psyche, or rather one who has committed himself to being one, must do. Reason is the organ of any recognition and

ought to be developed not only by a physician but by all men. This in fact happens very seldom, which explains why our knowledge and our actions are so often blundering. Whoever does not live in light lives in darkness (and a deceptive light, say, the light of a false philosophy, is also darkness), and it is the purpose of the doctor of the soul to bring the mentally disturbed, whose inner life is totally darkened, back to light. But how can he do this if he himself does not live in light? It is necessary to sharply emphasize this point of view of the doctor of the soul. Whoever cannot make this point of view his own must give up the name and the power and the business of a doctor of the soul.

Thus, the doctor of the soul (or psyche) is a true man of reason. He has overcome his selfish interests and treats for purely humanitarian reasons. He considers his patients only as sufferers and not in relation to his own personality. Much is gained even by this attitude alone, since in this manner he obtains an unprejudiced, correct view. He does not hold the vulgar and limited view according to which it is the bodily relationships which determine both the disturbance and the cure, but will concentrate on the soul life and will view all diseased manifestations of the psyche in relation to the latter. From the very outset he influences the patient by virtue of his, one may be permitted to say, holy presence, by the sheer strength of his being, his glance and his will. The will exists in man as a force which is not cultivated; it is however the will which gives rise to all creation, and man, too, has his share of this creative force. The will is the principle of miracles, the principle of magnetism. The magnetic manipulation is only an ad hoc device, a kind of mechanical stimulant of the will. But will without spirit is blind, and will without temperament is barren. The man of reason combines all forces of his inner being for a full understanding and for the living deed. *Sapere aude*!*

From Johann Christian August Heinroth, *Textbook of Disturbances of Mental Life; or, Disturbances of the Soul and Their Treatment*, translated by J. Schmorak, introduction by George Mora, vol. 1, *Theory* (Baltimore: Johns Hopkins University Press, 1975), 19–29. © The Johns Hopkins University Press. Reprinted with permission of The Johns Hopkins University Press.

*Editor's note: "Dare to know!"

Jean Etienne Esquirol
(1772–1840)

"Monomania"
(1838)

Esquirol was a student of Philippe Pinel, whom he succeeded as director at the Salpêtrière Hospital in Paris in 1811. Expanding on Pinel's thinking, Esquirol believed that dealing with madness required a host of social and legal reforms, and he eventually became the chief architect of France's modern asylum system. Enamored with nosology (the science of disease classification), one of his legacies was to develop the diagnosis of monomania. Up until the turn of the nineteenth century, physicians, legislators, and courts tended to see insanity as a defect of reason, namely, the problem rested in the individual's inability to rationally comprehend. Esquirol's innovation was to argue that a person's emotions or will power could be impaired, without affecting his or her ability to reason. The diagnosis of monomania was an attempt to come to grips with this possibility. In the decades that followed, it provided the inspiration for concepts such as obsession and psychopathy.

Monomania

After having set forth characteristics of lypemania (melancholy with delirium), it becomes my duty to describe that form of partial delirium, to which I have given the name monomania; but first, I will endeavor to point out the distinctive characteristics of those two forms of delirium. Monomania and lypemania, are chronic cerebral affections, unattended by fever, and characterized by a partial lesion of the intelligence, affections, or will. At one time, the intellectual disorder is confined to a single object, or a limited number of objects. The patients seize upon a false principle, which they pursue without deviating from logical reasonings, and from which they deduce legitimate consequences, which modify their affections, and the acts of their

will. Aside from this partial delirium, they think, reason and act, like other men. Illusions, hallucinations, vicious associations of ideas, false and strange convictions, are the basis of this delirium, which I would denominate, *intellectual monomania*. At another; monomaniacs are not deprived of the use of their reason, but their affections and dispositions are perverted. By plausible motive, by very reasonable explanations, they justify the actual condition of their sentiments, and excuse the strangeness and inconsistency of their conduct. It is this, which authors have called *reasoning mania*, but which I would name *affective monomania*.

In a third class of cases, a lesion of the will exists. The patient is drawn away from his accustomed course, to the commission of acts, to which neither reason nor sentiment determine, which conscience rebukes, and which the will has no longer the power to restrain. The actions are involuntary, instinctive, irresistible. This is *monomania without delirium*, or, *instinctive monomania*. . . .

M. H., forty-five years of age, a bachelor, and counsellor-at-law, is of a medium stature, bilious-sanguine temperament, of an excellent constitution, and has a remarkably voluminous head. His forehead is uncovered to a very considerable extent, his hair black, his eyes full of vivacity, and his complexion swarthy. He has always led a regular life, and conducted his affairs with system and integrity. He resided for some time at Guadeloupe, was sick for a year after a struggle with the climate, in connection with reverses of fortune, was sent back to Paris, and admitted to Charenton, Nov. 20th, 1832. During the first months of his sojourn at this establishment, he appears composed, walking in the garden, reading much, and conversing with spirit. He would have been regarded as rational, if, from time to time, his delirium did not make itself manifest. He called himself the son of Louis XVI, and was accustomed to add, that an attempt had been made to poison him, for political purpose. After some months, the delirium manifests itself more habitually, and at length reaches a state of fury. He is king, and as such, expects to command and to be obeyed. Those who surround him are his slaves, and their right to life and death is vested in him. Wo to the man who accosts him, without recognizing his kingly power. A doubt on this point, is high treason. The domestics who serve him, know full well the precautions which it is necessary to take, in presenting themselves to him. In several instances, his threats and transports of passion, on my endeavoring to combat his error, have put me on my guard.

In his case, every circumstance comports with his conviction. His lofty carriage, his attitude and look; the imperious tone of his voice and gestures, most clearly express the vain prejudices that occupy his mind. He does not adorn himself with the insignia of his rank, and with ribbons, after the manner of monomaniac *kings* with whom we meet among assemblages of the insane; but the walls of his cell, which he regards as a dungeon, present, written in large characters, both words and phrases, which disclose his mental condition. Observe some of the inscriptions, which he has traced in the form of letters, as they stand upon the walls:

I have–. Tuesday–. *A rabble of Frenchmen*–. Farther on: *Mortal hatred to the French Nation*,– to the people, to the Nobility–by S.A.R., Prince of Bourbon, etc. April 1st, 1837: Son of Louis XVI.– King. Below: I am not a *Man*—but a Prince—King—MONARCH.

This hatred of the French, and these titles which he proclaims with pride, constitute the subject of all his letters and writings. He feels indignant at the injustice that restrains him by prison bolts; so great and powerful as he is. He pretends that they have taken possession of him by supernatural means, which spies,—selected from the most degraded of the French people—employ; *by pouring upon his majesty, torrents of electricity, in order to annihilate him*. Sometimes he refuses food, not wishing to be nourished like the clowns of his corridor. His food ought to be prepared in royal kitchens. His grandeur and power permit him to recognize, as his relatives and friends, none other than the Bourbons, Ferdinands, Nicholases, etc. . . .

Monomaniacs, like other insane persons, are subject to illusions and hallucinations, which often alone characterize their delirium, and are the causes of the perversion of their affections, and the disorder of their actions. Numerous facts justify this statement. Transported by enthusiasm, by religious or political fanaticism, warmed by erotic passions, blinded by notions of an imaginary good fortune, flattered by sentiments of a felicity of which they deem themselves alone worthy, monomaniacs entertain little affection for their relatives and friends, or their tenderness is exaggerated. They often disdain persons whom they are accustomed to love most tenderly, and feel a sort of pity for them, in consequence of their pretended ignorance, or supposed poverty, or because they are unworthy of understanding the good fortune of the monomaniac, or of participating in it. Like all insane persons, these patients neglect their own interest and affairs, and treat with contempt the usages of society. There are insane persons in conformity with

the strictest principles, remarkable for the rectitude of their understanding, for the delicacy of their sentiments, for the mildness of their dispositions, and for a uniformly sober and moral life; who, in consequence of some physical or moral causes, change their disposition and habits of conduct, become turbulent and unsociable, and perform odd, singular, culpable, and sometimes dangerous acts, in opposition to their affections and interests. A partial lesion of the understanding causes these changes, and perverts the sentiments and actions of this class of patients. Thus, the old man, who believes that he hears the voice of an angel, who commands him to offer up his son, after the example of Abraham, and perform this sacrifice, is a monomaniac. . . .

The causes which predispose and produce monomania, are the same with those which produce insanity in general. Sanguine and nervous-sanguine temperaments, and persons endowed with a brilliant, warm and vivid imagination; minds of a meditative and exclusive cast, which seem to be susceptible only of a series of thoughts and emotions; individuals who, through self-love, vanity, pride and ambition, abandon themselves to their reflections, to exaggerated projects and unwarrantable pretensions, are especially disposed to monomania. It is remarkable, that these individuals almost invariably beguile themselves with the hope of a happy fortune, when, stricken by some reverse, or disappointed in their lofty expectations, they fall sick. Thus, a man who is actually happy, and moderate in his desires, and who, by some exciting cause, becomes insane, will not be a monomaniac; whilst an ambitious, proud, or amorous man, who shall have become unfortunate, or have lost the object of his affections, will. It would seem as if monomania were only an exaggeration of the thoughts, desires and illusions with respect to the future, with which these unfortunate beings amuse their fancy, previous to their illness.

A weak understanding, little cultivated or developed; and the want, or vices of education, also predispose to monomania. The exciting causes are; errors of regimen, strong passions, and especially reverses of fortune, disappointed self-love, or ambition. Religious excitement also, ascetic meditations and the reading of romances, often produce this disease among those who are essentially controlled by pride and vanity. . . . Monomania is remittent or intermittent; and the symptoms are exasperated, particularly at the menstrual periods. It is sometimes preceded by melancholy and lypemania, and is complicated with epilepsy, hysteria, hypochondria, and very fre-

quently with paralysis. The progress of monomania is rapid and violent. Its termination is often unexpected, and is effected like other forms of mental alienation, by crises, more or less sensible. But it not unfrequently terminates suddenly, without cause, or perceptible crisis, or by a vivid moral impression. Monomania sometimes passes into mania, and sometimes alternates with lypemania. When prolonged, it degenerates into dementia. . . . But when the disease degenerates into a chronic state, the monomaniac is not only irrational in his hypothesis, but his reasonings, affections and acts, which were previously, the proper consequences of the idea or controlling affection of his mind, no longer maintain their logical and natural connection. . . . The treatment of monomania should, as in other forms of mental alienation, be directed with a special reference to the predisposing and exciting causes of the disease, and to the physical disorders. The intellectual and moral symptoms should have great weight in the therapeutic views of the physician. In this malady, which is characterized by a peculiarly nervous condition of the system, antispasmodics are very useful. While we may, with advantage, have recourse to hygienic agents, it is proper also, to hope for success from moral treatment. Here, more than in other forms of mental disease, and with better hopes of success, we apply to the understanding and passions of the patient, with a view to effect his cure. We have recourse to surprises, subterfuges, and oppositions, ingeniously managed, as circumstances suggest, the genius of the physician gives birth to, and as experience may hit upon, and appropriately pursue.

From E. Esquirol, *Mental Maladies: A Treatise on Insanity* (Philadelphia: Lea and Blanchard, 1845), 320, 327–328, 333–334.

Dorothea Dix

(1802–1887)

Memorial to the Legislature of Massachusetts

(1843)

Dorothea Dix was one of the early American activists for prisoners and the insane poor. The daughter of an itinerant preacher, she had the opportunity in 1836 to travel to England, where she became familiar with the ideas of the prison reformer Elizabeth Frye (1780–1845) and the reform-minded asylum director Samuel Tuke (1784–1857). After returning to the United States, she happened to examine accommodations for the insane at the Cambridge, Massachusetts, jail and was appalled to find the inmates there chained up in dungeon cells. She then set about touring poorhouses, jails, and prisons to investigate conditions elsewhere, first traveling from county to county, then from state to state. Her findings led her to publicly advocate for the reform of state institutions of confinement. In the 1850s, she took her mission overseas to Britain, France, Greece, Russia, and Japan. All in all, Dix proved to be one of the most internationally respected and effective reformers of the nineteenth century. The following excerpt comes from her 1843 appeal to the state legislature of Massachusetts.

GENTLEMEN.

I respectfully ask to present this Memorial, believing that the *cause*, which actuates to and sanctions so unusual a movement, presents no equivocal claim to public consideration and sympathy. Surrendering to calm and deep convictions of duty my habitual views of what is womanly and becoming, I proceed briefly to explain what has conducted me before you unsolicited and unsustained, trusting, while I do so, that the memorialist will be speedily forgotten in the memorial.

About two years since leisure afforded opportunity, and duty prompted me to visit several prisons and alms-houses in the vicinity of this metropolis. I found, near Boston, in the Jails and Asylums for the poor, a numerous class brought into unsuitable connexion with criminals and the general mass of Paupers. I refer to Idiots and Insane persons, dwelling in circumstances not only adverse to their own physical and moral improvement, but productive of extreme disadvantages to all other persons brought into association with them. I applied myself diligently to trace the causes of these evils, and sought to supply remedies. As one obstacle was surmounted, fresh difficulties appeared. Every new investigation has given depth to the conviction that it is only by decided, prompt, and vigorous legislation the evils to which I refer, and which I shall proceed more fully to illustrate, can be remedied. I shall be obliged to speak with great plainness, and to reveal many things revolting to the taste, and from which my woman's nature shrinks with peculiar sensitiveness. But truth is the highest consideration. *I tell what I have seen*—painful and shocking as the details often are—that from them you may feel more deeply the imperative obligation which lies upon you to prevent the possibility of a repetition or continuance of such outrages upon humanity. If I inflict pain upon you, and move you to horror, it is to acquaint you with sufferings which you have the power to alleviate, and make you hasten to the relief of the victims of legalized barbarity.

I come to present the strong claims of suffering humanity. I come to place before the Legislature of Massachusetts the condition of the miserable, the desolate, the outcast. I come as the advocate of helpless, forgotten, insane and idiotic men and women, sunk to a condition from which the most unconcerned would start with real horror; of beings wretched in our Prisons, and more wretched in our Alms-Houses. And I cannot suppose it needful to employ earnest persuasion, or stubborn argument, in order to attest and fix attention upon a subject, only the more strongly pressing in its claims, because it is revolting and disgusting in its details.

I must confine myself to few examples, but am ready to furnish other and more complete details, if required. If my pictures are displeasing, coarse, and severe, my subjects, it must be recollected, offer no tranquil, refined, or composing features. The condition of human beings, reduced to the extremest states of degradation and misery, cannot be exhibited in softened language, or adorn a polished page.

I proceed, Gentlemen, briefly to call your attention, to the *present* state of Insane Persons confined within this Commonwealth, in *cages, closets, cellars, stalls, pens*! *Chained, naked, beaten with rods*, and *lashed* into obedience!

As I state cold, severe *facts*, I feel obliged to refer to persons, and definitely to indicate localities. But it is upon my subject, not upon localities or individuals, I desire to fix attention; and I would speak as kindly as possible of all Wardens, Keepers, and other responsible officers, believing that *most* of these have erred not through hardness of heart and willful cruelty, so much as want of skill and knowledge, and want of consideration. Familiarity with suffering, it is said, blunts the sensibilities, and where neglect once finds a footing other injuries are multiplied. This is not all, for it may justly and strongly be added that, from the deficiency of adequate means to meet the wants of these cases, it has been an absolute impossibility to do justice in this matter. Prisons are not constructed in view of being converted into County Hospitals, and Alms-Houses are not constructed as receptacles for the Insane. And yet, in the face of justice and common sense, Wardens are by law compelled to receive, and the Masters of Alms-Houses not to refuse, Insane and Idiotic subjects in all stages of mental disease and privation.

It is the Commonwealth, not its integral parts, that is accountable for most of the abuses which have lately, and do still exist. I repeat it, it is defective legislation which perpetuates and multiplies these abuses.

In illustration of my subject, I offer the following extracts from my Note-Book and Journal. . . .

The use of cages all but universal; hardly a town but can refer to some not distant period of using them; chains are less common; negligences frequent; willful abuse less frequent than sufferings proceeding from ignorance, or want of consideration. I encountered during the last three months many poor creatures wandering reckless and unprotected through the country. Innumerable accounts have been sent me of persons who had roved away unwatched and unsearched after; and I have heard that responsible persons, controlling the almshouses, have not thought themselves culpable in sending away from their shelter, to cast upon the chances of remote relief, insane men and women. These, left on the highways, unfriended and incompetent to control or direct their own movements, sometimes have found refuge in the hospital, and others have not been traced. But I cannot particularize; in traversing the state I have found hundreds of insane person in every variety of circumstance and condition;

many whose situation could not and need not be improved; a less number, but very large, whose lives are the saddest pictures of human suffering and degradation. I give a few illustrations; but description fades before reality.

Danvers. November; visited the almshouse; a large building, much out of repair; understand a new one is in contemplation. Here are fifty-six to sixty inmates; one idiotic; three insane; one of the latter in close confinement at all times.

Long before reaching the house, wild shouts, snatches of rude songs, imprecations, and obscene language, fell upon the ear, proceeding from the occupant of a low building, rather remote from the principal building to which my course was directed. Found the mistress, and was conducted to the place, which was called "*the home*" of the *forlorn* maniac, a young woman, exhibiting a condition of neglect and misery blotting out the faintest idea of comfort, and outraging every sentiment of decency. She had been, I learnt, "a respectable person; industrious and worthy; disappointments and trials shook her mind, and finally laid prostrate reason and self-control; she became a maniac for life! She had been at Worcester Hospital for a considerable time, and had been returned as incurable." The mistress told me she understood that, while there, she was "comfortable and decent." Alas! what a change was here exhibited! She had passed from one degree of violence and degradation to another, in swift progress; there she stood, clinging to, or beating upon, the bars of her caged apartment, the contracted size of which afforded space only for increasing accumulations of filth, a *foul* spectacle; there she stood with naked arms and disheveled hair; the unwashed frame invested with fragments of unclean garments, the air so extremely offensive, though ventilation was afforded on all sides save one, that it was not possible to remain beyond a few moments without retreating for recovery to the outward air. Irritation of body, produced by utter filth and exposure, incited her to the horrid process of tearing off her skin by inches; her face, neck, and person, were thus disfigured to hideousness; she held up a fragment just rent off; to my exclamation of horror, the mistress replied, "Oh, we can't help it; half the skin is off sometimes; we can do nothing with her; and it makes no difference what she eats, for she consumes her own filth as readily as the food which is brought her."

It is now January; a fortnight since, two visitors reported that most wretched outcast as "wallowing in dirty straw, in a place yet more dirty, and without clothing, without fire. Worse cared for than the brutes, and wholly

lost to consciousness of decency!" Is the whole story told? What was seen is; what is reported is not. These gross exposures are not for the pained sight of one alone; all, all, coarse, brutal men, wondering, neglected children, old and young, each and all, witness the lowest, foulest state of miserable humanity. And who protects her, that worse than Paria outcast, from other wrongs and blacker outrages? I do not *know* that such *have been*. I do know that they are to be dreaded, and that they are not guarded against.

Some may say these things cannot be remedied; these furious maniacs are not to be raised from these base conditions. I *know* they are; could give *many* examples; let *one* suffice. A young woman, a pauper, in a distant town, *Sandisfield*, was for years a raging maniac. A cage, chain, and *the whip*, were the agents for controlling her, united with harsh tones and profane language. Annually, with others (the town's poor) she was put up at auction, and bid off at the lowest price which was declared for her. One year, not long past, an old man came forward in the number of applicants for the poor wretch; he was taunted and ridiculed; "what would he and his old wife do with such a mere beast?" "My wife says yes," replied he, "and I shall take her." She was given to his charge; he conveyed her home; she was washed, neatly dressed, and placed in a decent bed-room, furnished for comfort and opening into the kitchen. How altered her condition! As yet *the chains* were not off. The first week she was somewhat restless, at times violent, but the quiet kind ways of the old people wrought a change; she received her food decently; forsook acts of violence, and no longer uttered blasphemous or indecent language; after a week, the chain was lengthened, and she was received as a companion into the kitchen. Soon she engaged in trivial employments. "After a fortnight," said the old man, "I knocked off the chains and made her a free woman." She is at times excited, but not violently; they are careful of her diet; they keep her very clean; she calls them "father" and "mother." Go there now and you will find her "clothed," and though not perfectly in her "right mind," so far restored as to be a safe and comfortable inmate.

Newburyport. Visited the almshouse in June last; eighty inmates; seven insane, one idiotic. Commodious and neat house; several of the partially insane apparently very comfortable; two very improperly situated, namely, an insane man, not considered incurable, in an out-building, whose room opened upon what was called "the dead room," affording in lieu of companionship with the living, a contemplation of corpses! The other subject

was a woman in a *cellar*. I desired to see her; much reluctance was shown. I pressed the request; the Master of the House stated that she was *in the cellar*; that she was *dangerous to be approached*; that "she had lately attacked his wife"; and *was often naked*. I persisted: "if you will not go with me, give me the keys and I will go alone." Thus importuned, the outer doors were opened. I descended the stairs from within; a strange, unnatural noise seemed to proceed from beneath our feet; at the moment I did not much regard it. My conductor proceeded to remove a padlock while my eye explored the wide space in quest of the poor woman. All for a moment was still. But judge my horror and amazement, when a door to a closet *beneath* the *staircase* was opened, revealing in the imperfect light a female apparently wasted to a skeleton, partially wrapped in blankets, furnished for the narrow bed on which she was sitting; her countenance furrowed, not by age, but suffering, was the image of distress; in that contracted space, unlighted, unventilated, she poured forth the wailings of despair; mournfully she extended her arms and appealed to me, "why am I consigned to hell? dark—dark—I used to pray, I used to read the Bible—I have done no crime in my heart; I had friends, why have all forsaken me!—my God! my God! why hast *thou* forsaken me!" Those groans, those wailings come up daily, mingling with how many others, a perpetual and sad memorial. When the good Lord shall require an account of stewardship, what shall all and each answer!

Perhaps it will be inquired how long, how many days or hours she was imprisoned in these confined limits? *For years!* In another part of the cellar were other small closets, only better, because higher through the entire length, into one of which she by turns was transferred, so as to afford opportunity for fresh whitewashing, &c. . . .

Violence and severity do but exasperate the Insane: the only availing influence is kindness and firmness. It is amazing what these will produce. How many examples might illustrate this position: I refer to one recently exhibited in Barre. The town Paupers are disposed of annually to some family who, for a stipulated sum agree to take charge of them. One of them, a young woman, was shown to me well clothed, neat, quiet, and employed at needle-work. It is possible that this is the same being who, but last year, was a raving madwoman, exhibiting every degree of violence in action and speech; a very tigress wrought by fury; caged, chained, beaten, loaded with injuries, and exhibiting the passions which an iron rule might be expected

to stimulate and sustain. It is the same person; another family hold her in charge who better understand human nature and human influences; she is no longer chained, caged, and beaten; but if excited, a pair of mittens drawn over the hands secures from mischief. Where will she be next year, after the annual sale?

. . . I may here remark that severe measures, in enforcing rule, have in many places been openly revealed. I have not seen chastisement administered by stripes, and in but few instances have I seen the *rods* and *whips*, but I have seen blows inflicted, both passionately and repeatedly.

I have been asked if I have investigated the causes of insanity? I have not; but I have been told that this most calamitous overthrow of reason, often is the result of a life of sin; it is sometimes, but rarely added, they must take the consequences; they deserve no better care! Shall man be more just than God; he who causes his sun, and refreshing rains, and life-giving influence, to fall alike on the good and the evil? Is not the total wreck of reason, a state of distraction, and the loss of all that makes life cherished a retribution, sufficiently heavy, without adding to consequences so appalling, every indignity that can bring still lower the wretched sufferer? Have pity upon those who, while they were supposed to lie hid in secret sins, "have been scattered under *a dark veil of forgetfulness*; over whom is spread a heavy night, and who unto themselves are more grievous than the darkness."

. . . We need an Asylum for this class, the incurable, where conflicting duties shall not admit of such examples of privations and misery.

One is continually amazed as the tenacity of life in these persons. In conditions that wring the heart to behold, it is hard to comprehend that days rather than years should not conclude the measure of their griefs and miseries. Picture her condition! place yourselves in that dreary cage, remote from the inhabited dwelling, alone by day and night, without fire, without clothes, *except when remembered*; without object or employment; weeks and months passing on in drear succession, not a blank, but with keen life to suffering; with kindred, but deserted by them; and you shall not lose the memory of that time when they loved you, and you in turn loved them, but now no act or voice of kindness makes sunshine in the heart. Has fancy realized this to you? It *may* be the state of some of those you cherish! Who shall be sure his own hearth-stone shall not be desolate? nay, who shall say his own mountain stands strong, his lamp of reason shall not go out in

darkness! To show how many has this become a heart-rending reality! If for selfish ends only, should not effectual Legislation here interpose?

. . . Men of Massachusetts, I beg, I implore, I demand, pity and protection, for these of my suffering, outraged sex!—Fathers, Husbands, Brothers, I would supplicate you for this boon—but what do I say? I dishonor you, divest you at once of christianity and humanity—does this appeal imply distrust. If it comes burthened with a doubt of your righteousness in this Legislation, then blot it out; while I declare confidence in your honor, not less than your humanity. Here you will put away the cold, calculated spirit of selfishness and self-seeking; lay off the armor of local strife and political opposition; here and now, for once, forgetful of the earthly and perishable, come up to these halls and consecrate them with one heart and one mind to works of righteousness and just judgment. Become the benefactors of your race, the just guardians of the solemn rights you hold in trust. Raise up the fallen; succor the desolate; restore the outcast; defend the helpless; and for your eternal and great reward, receive the benediction . . . "Well done, good and faithful servants, become rulers over many things!"

From Dorothea Dix, *Memorial* (Boston: Munroe & Francis, 1843), 3–9, 12, 17–18, 20–21, 24–25.

The M'Naughten Rules

(1843)

Up until the early nineteenth century, British and American courts had no universally accepted standard for defining mental incompetence in criminal cases. In general, judges tended to focus on the accused's intellectual capacity for understanding right and wrong. The growing emphasis in psychiatry on impairments of emotion and will, however, seemed to challenge this more or less cognitive notion of insanity.

Daniel M'Naughten (1813–1865) was a Scottish craftsman, active in the early workers' movement in Great Britain. On 20

January 1843, believing that conservatives were intent on murdering him, M'Naughten set out to stalk and kill the sitting prime minister, Robert Peel. Mistaking Peel's secretary Edward Drummond for the government leader, M'Naughten shot Drummond with a pistol at point-blank range. Drummond later died. At his trial in March, M'Naughten pleaded not guilty, his legal counsel enlisting the expert testimony of physicians, who argued that M'Naughten suffered from a "moral insanity" in the form of monomania. The strategy succeeded, and M'Naughten was found not guilty by reason of insanity and confined to an asylum for the rest of his life.

Queen Victoria, displeased with the verdict, asked the House of Lords to review the decision with a panel of judges. In June 1843, the House of Lords demanded that the judges answer several abstract questions involving issues raised by the case. The judges' responses have since become known as the M'Naughten Rules, and they have served as the basis for determining legal insanity throughout many parts of England and the United States to this very day.

Notwithstanding a party accused did an act, which was in itself criminal, under the influence of insane delusion, with a view of redressing or revenging some supposed grievance or injury, or of producing some public benefit, he is nevertheless punishable if he knew at the time that he was acting contrary to law.

That if the accused was conscious that the act was one which he ought not to do; and if the act was at the same time contrary to law, he is punishable. In all cases of this kind the jurors ought to be told that every man is presumed to be sane, and to possess a sufficient degree of reason to be responsible for his crimes, until the contrary be proved to their satisfaction: and that to establish a defence on the ground of insanity, it must be clearly proved that at the time of commiting the act the party accused was labouring under such a defect of reason, from disease of the mind, as not to know the nature and quality of the act he was doing, or as not to know that what he was doing was wrong.

That a party labouring under a partial delusion must be considered in the same situation, as to responsibility, as if the facts, in respect to which the delusion exists, were real.

That where an accused person is supposed to be insane, a medical man, who has been present in Court and heard the evidence, may be asked, as a matter of science, whether the facts stated by the witnesses, supposing them to be true, show a state of mind incapable of distinguishing between right and wrong.

The prisoner had been indicted for that he, on the 20th day of January 1843, at the parish of Saint Martin in the Fields, in the county of Middlesex, and within the jurisdiction of the Central Criminal Court, in and upon one Edward Drummond, feloniously, wilfully, and of his malice aforethought, did make an assault; and that the said Daniel M'Naghten,* a certain pistol of the value of 20s, loaded and charged with gunpowder and a leaden bullet (which pistol he in his right hand had and held), to, against and upon the said Edward Drummond, feloniously, wilfully, and of his malice aforethought, did shoot and discharge; and that the said Daniel M'Naghten, with the leaden bullet aforesaid, out of the pistol aforesaid, by force of the gunpowder, etc., the said Edward Drummond, in and upon the back of him the said Edward Drummond, feloniously, etc. did strike, penetrate and wound, giving to the said Edward Drummond, in and upon the back of the said Edward Drummond, one mortal wound, etc., of which mortal wound the said E. Drummond languished until the 25th of April and then died; and that by the means aforesaid, he the prisoner did kill and murder the said Edward Drummond. The prisoner pleaded Not guilty.

Evidence having been given of the fact of the shooting of Mr. Drummond, and of his death in consequence thereof, witnesses were called on the part of the prisoner, to prove that he was not, at the time of committing the act, in a sound state of mind. The medical evidence was in substance this: That persons of otherwise sound mind, might be affected by morbid delusions: that the prisoner was in that condition: that a person so labouring under a morbid delusion, might have a moral perception of right and wrong, but that in the case of the prisoner it was a delusion which carried him away beyond the power of his own control, and left him no such perception; and that he was not capable of exercising any control over acts which had connexion with his delusion: that it was of the nature of the disease with which the prisoner was affected, to go on gradually until it had

*Editor's note: M'Naughten's surname was spelled a variety of ways in public documents.

reached a climax, when it burst forth with irresistible intensity: that a man might go on for years quietly, though at the same time under its influence, but would all at once break out into the most extravagant and violent paroxysms.

Some of the witnesses who gave this evidence, had previously examined the prisoner: others had never seen him till he appeared in Court, and they formed their opinions on hearing the evidence given by the other witnesses.

Lord Chief Justice Tindal (in his charge): The question to be determined is, whether at the time the act in question was committed, the prisoner had or had not the use of his understanding, so as to know that he was doing a wrong or wicked act. If the jurors should be of opinion that the prisoner was not sensible, at the time he committed it, that he was violating the laws both of God and man, then he would be entitled to a verdict in his favour: but if, on the contrary, they were of opinion that when he committed the act he was in a sound state of mind, then their verdict must be against him.

Verdict, Not guilty, on the ground of insanity.

This verdict, and the question of the nature and extent of the unsoundness of mind which would excuse the commission of a felony of this sort, having been made the subject of debate in the House of Lords (the 6th and 13th March 1843; see Hansard's Debates, vol. 67, pp. 288, 714), it was determined to take the opinion of the Judges on the law governing such cases. Accordingly, on the 26th of May, all the Judges attended their Lordships, but no questions were then put.

On the nineteenth of June, the Judges again attended the House of Lords; when (no argument having been had) the following questions of law were propounded to them:

1st. What is the law respecting alleged crimes committed by persons afflicted with insane delusion, in respect of one or more particular subjects or persons: as, for instance, where at the time of the commission of the alleged crime, the accused knew he was acting contrary to law, but did the act complained of with a view, under the influence of insane delusion, of redressing or revenging some supposed grievance or injury, or of producing some supposed public benefit?

2d. What are the proper questions to be submitted to the jury, when a person alleged to be afflicted with insane delusion respecting one or more

particular subjects or persons, is charged with the commission of a crime (murder, for example), and insanity is set up as a defence?

3d. In what terms ought the question to be left to the jury, as to the prisoner's state of mind at the time when the act was committed?

4th. If a person under an insane delusion, as to existing facts, commits an offence in consequence thereof, is he thereby excused?

5th. Can a medical man conversant with the disease of insanity, who never saw the prisoner previously to the trial, but who was present during the whole trial and the examination of all the witnesses, be asked his opinion as to the state of the prisoner's mind at the time of the commission of the alleged crime, or his opinion whether the prisoner was conscious at the time of doing the act, that he was acting contrary to law, or whether he was labouring under any and what delusion at the time?

Mr. Justice Maule: I feel great difficulty in answering the questions put by your Lordships on this occasion: First, because they do not appear to arise out of and are not put with reference to a particular case, or for a particular purpose, which might explain or limit the generality of their terms, so that full answers to them ought to be applicable to every possible state of facts, not inconsistent with those assumed in the questions: this difficulty is the greater, from the practical experience both of the bar and the Court being confined to questions arising out of the facts of particular cases: Secondly, because I have heard no argument at your Lordships' bar or elsewhere, on the subject of these questions; the want of which I feel the more, the greater are the number and extent of questions which might be raised in argument: and Thirdly, from a fear of which I cannot divest myself, that as these questions relate to matters of criminal law of great importance and frequent occurrence, the answers to them by the Judges may embarrass the administration of justice, when they are cited in criminal trials. For these reasons I should have been glad if my learned brethren would have joined me in praying your Lordships to excuse us from answering these questions; but as I do not think they ought to induce me to ask that indulgence for myself individually, I shall proceed to give such answers as I can, after the very short time which I have had to consider the questions, and under the difficulties I have mentioned; fearing that my answers may be as little satisfactory to others as they are to myself.

The first question, as I understand it, is, in effect, What is the law respecting the alleged crime, when at the time of the commission of it, the

accused knew he was acting contrary to the law, but did the act with a view, under the influence of insane delusion, of redressing or revenging some supposed grievance or injury, or of producing some supposed public benefit? If I were to understand this question according to the strict meaning of its terms, it would require, in order to answer it, a solution of all questions of law which could arise on the circumstances stated in the question, either by explicitly stating and answering such questions, or by stating some principles or rules which would suffice for their solution. I am quite unable to do so, and, indeed, doubt whether it be possible to be done; and therefore request to be permitted to answer the question only so far as it comprehends the question, whether a person, circumstanced as stated in the question, is, for that reason only, to be found not guilty of a crime respecting which the question of his guilt has been duly raised in a criminal proceeding, and I am of opinion that he is not. There is no law, that I am aware of, that makes persons in the state described in the question not responsible for their criminal acts. To render a person irresponsible for crime on account of unsoundness of mind, the unsoundness should, according to the law as it has long been understood and held, be such as rendered him incapable of knowing right from wrong. The terms used in the question cannot be said (with reference only to the usage of language) to be equivalent to a description of this kind and degree of unsoundness of mind. If the state described in the question be one which involves or is necessarily connected with such an unsoundness, this is not a matter of law but of physiology, and not of that obvious and familiar kind as to be inferred without proof.

Second, the questions necessarily to be submitted to the jury, are those questions of fact, which are raised on the record. In a criminal trial, the question commonly is, whether the accused be guilty or not guilty: but, in order to assist the jury in coming to a right conclusion on this necessary and ultimate question, it is usual and proper to submit such subordinate or intermediate questions, as the course which the trial has taken may have made it convenient to direct their attention to. What those questions are, and the manner of submitting them, is a matter of discretion for the Judge: a discretion to be guided, by a consideration of all the circumstances attending the inquiry. In performing this duty, it is sometimes necessary or convenient to inform the jury as to the law; and if, on a trial such as is suggested in the question, he should have occasion to state what kind and

degree of insanity would amount to a defence, it should be stated conformably to what I have mentioned in my answer to the first question, as being, in my opinion, the law on this subject.

Third, there are no terms which the Judge is by law required to use. They should not be inconsistent with the law as above stated, but should be such as, in the discretion of the Judge, are proper to assist the jury in coming to a right conclusion as to the guilt of the accused.

Fourth, the answer which I have given to the first question, is applicable to this.

Fifth, whether a question can be asked depends not merely on the questions of fact raised on the record, but on the course of the cause at the time it is proposed to ask it; and the state of an inquiry as to the guilt of a person charged with a crime, and defended on the ground of insanity, may be such, that such a question as either of those suggested, is proper to be asked and answered, though the witness has never seen the person before the trial, and though he has merely been present and heard the witnesses: these circumstances, of his never having seen the person before, and of his having merely been present at the trial, not being necessarily sufficient, as it seems to me, to exclude the lawfulness of a question which is otherwise lawful; though I will not say that an inquiry might not be in such a state, as that these circumstances should have such an effect.

Supposing there is nothing else in the state of the trial to make the questions suggested proper to be asked and answered, except that the witness had been present and heard the evidence; it is to be considered whether that is enough to sustain the question. In principle it is open to this objection, that as the opinion of the witness is founded on those conclusions of fact which he forms from the evidence, and as it does not appear what those conclusions are, it may be that the evidence he gives is on such an assumption of facts, as makes it irrelevant to the inquiry. But such questions have been very frequently asked, and the evidence to which they are directed has been given, and has never, that I am aware of, been successfully objected to. Evidence, most clearly open to this objection, and on the admission of which the event of a most important trial probably turned, was received in the case of The Queen v. M'Naghten, tried at the Central Criminal Court in March last, before the Lord Chief Justice, Mr. Justice Williams, and Mr. Justice Coleridge, in which counsel of the highest eminence were engaged on both sides; and I think the course and practice of

receiving such evidence, confirmed by the very high authority of these Judges, who not only received it, but left it, as I understand, to the jury, without any remark derogating from its weight, ought to be held to warrant its reception, notwithstanding the objection in principle to which it may be open. In cases even where the course of practice in criminal law has been unfavourable to parties accused, and entirely contrary to the most obvious principles of justice and humanity, as well as those of law, it has been held that such practice constituted the law, and could not be altered without the authority of Parliament.

Lord Chief Justice Tindal: My Lords, Her Majesty's Judges (with the exception of Mr. Justice Maule, who has stated his opinion to your Lordships), in answering the questions proposed to them by your Lordships' House, think it right, in the first place, to state that they have forborne entering into any particular discussion upon these questions, from the extreme and almost insuperable difficulty of applying those answers to cases in which the facts are not brought judicially before them. The facts of each particular case must of necessity present themselves with endless variety, and with every shade of difference in each case; and as it is their duty to declare the law upon each particular case, on facts proved before them, and after hearing argument of counsel thereon, they deem it at once impracticable, and at the same time dangerous to the administration of justice, if it were practicable, to attempt to make minute applications of the principles involved in the answers given by them to your Lordships' questions.

They have therefore confined their answers to the statement of that which they hold to be the law upon the abstract questions proposed by your Lordships; and as they deem it unnecessary, in this peculiar case, to deliver their opinions seriatim, and as all concur in the same opinion, they desire me to express such their unanimous opinion to your Lordships.

The first question proposed by your Lordships is this: "What is the law respecting alleged crimes committed by persons afflicted with insane delusion in respect of one or more particular subjects or persons: as, for instance, where at the time of the commission of the alleged crime the accused knew he was acting contrary to law, but did the act complained of with a view, under the influence of insane delusion, of redressing or revenging some supposed grievance or injury, or of producing some supposed public benefit?"

In answer to which question, assuming that your Lordships' inquiries are confined to those persons who, labour under such partial delusions only, and are not in other respects insane, we are of opinion that, notwithstanding the party accused did the act complained of with a view, under the influence of insane delusion, of redressing or revenging some supposed grievance or injury, or of producing some public benefit, he is nevertheless punishable according to the nature of the crime committed, if he knew at the time of committing such crime that he was acting contrary to law; by which expression we understand your Lordships to mean the law of the land.

Your Lordships are pleased to inquire of us, secondly, "What are the proper questions to be submitted to the jury, where a person alleged to be afflicted with insane delusion respecting one or more particular subjects or persons, is charged with the commission of a crime (murder, for example), and insanity is set up as a defence?" And, thirdly, "In what terms ought the question to be left to the jury as to the prisoner's state of mind at the time when the act was committed?" And as these two questions appear to us to be more conveniently answered together, we have to submit our opinion to be, that the jurors ought to be told in all cases that every man is to be presumed to be sane, and to possess a sufficient degree of reason to be responsible for his crimes, until the contrary be proved to their satisfaction; and that to establish a defence on the ground of insanity, it must be clearly proved that, at the time of the committing of the act, the party accused was labouring under such a defect of reason, from disease of the mind, as not to know the nature and quality of the act he was doing; or, if he did know it, that he did not know he was doing what was wrong. The mode of putting the latter part of the question to the jury on these occasions has generally been, whether the accused at the time of doing the act knew the difference between, right and wrong: which mode, though rarely; if ever, leading to any mistake with the jury, is not, as we conceive, so accurate when put generally and in the abstract, as when put with reference to the party's knowledge of right and wrong in respect to the very act with which he is charged. If the question were to be put as to the knowledge of the accused solely and exclusively with reference to the law of the land, it might tend to confound the jury, by inducing them to believe that an actual knowledge of the law of the land was essential in order to lead to a conviction; whereas

the law is administered upon the principle that every one must be taken conclusively to know it, without proof that he does know it. If the accused was conscious that the act was one which he ought not to do, and if that act was at the same time contrary to the law of the land, he is punishable; and the usual course therefore has been to leave the question to the jury, whether the party accused had a sufficient degree of reason to know that he was doing an act that was wrong: and this course we think is correct, accompanied with such observations and explanations as the circumstances of each particular case may require.

The fourth question which your Lordships have proposed to us is this: "If a person under an insane delusion as to existing facts, commits an offence in consequence thereof, is he thereby excused?" To which question the answer must of course depend on the nature of the delusion: but, making the same assumption as we did before, namely, that he labours under such partial delusion only, and is not in other respects insane, we think he must be considered in the same situation as to responsibility as if the facts with respect to which the delusion exists were real. For example, if under the influence of his delusion he supposes another man to be in the act of attempting to take away his life, and he kills that man, as he supposes, in self-defence, he would be exempt from punishment. If his delusion was that the deceased had inflicted a serious injury to his character and fortune, and he killed him in revenge for such supposed injury, he would be liable to punishment.

The question lastly proposed by your Lordships is: "Can a medical man conversant with the disease of insanity, who never saw the prisoner previously to the trial, but who was present during the whole trial and the examination of all the witnesses, be asked his opinion as to the state of the prisoner's mind at the time of the commission of the alleged crime, or his opinion whether the prisoner was conscious at the time of doing the act that he was acting contrary to law, or whether he was labouring under any and what delusion at the time?" In answer thereto, we state to your Lordships, that we think the medical man, under the circumstances supposed, cannot in strictness be asked his opinion in the terms above stated, because each of those questions involves the determination of the truth of the facts deposed to, which it is for the jury to decide, and the questions are not mere questions upon a matter of science, in which case such evidence is admissible. But where the facts are admitted or not disputed, and the question

becomes substantially one of science only, it may be convenient to allow the question to be put in that general form, though the same cannot be insisted on as a matter of right.

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